

NanoGT Match Report: Cystic fibrosis

Disease: Cystic fibrosis (ORPHA:586)

Primary gene: CFTR

Gene CDS: 4443 bp

Inheritance: Autosomal recessive

Target tissues scored: lung, pancreas, liver

Interpretation

- At least one high-confidence precedent was found, but this is still a precedent match rather than a clinical-trial recommendation.
- Main review flags: Multi-system disease; define a primary therapeutic target tissue before selecting route/vector; Vector does not naturally cover all annotated disease tissues: pancreas; Cell-autonomous protein across multiple tissues; high transduction coverage may be required.

Disease Mechanism Evidence

Molecular mechanism: loss of function

Mechanistic detail: CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium

Gene-addition compatibility: conditional

Preferred modality class: airway gene addition

Evidence level/status: direct / needs_user_fact_check

Evidence summary: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges

Evidence source: OMIM CFTR 219700

Study-Level Limitations

- Catalog-relative ranking: current catalog contains 21 precedent programs and 8 vectors, so absence of a strong match is not proof that no therapy is possible.
 - Modality coverage is limited mainly to AAV and lentiviral precedents; dual-AAV, LNP/mRNA, genome editing, ASO, and transplant-enabling strategies are not fully represented.
 - Endpoint readiness: liver/metabolic targets may have biochemical biomarkers, but biomarker correction must be linked to clinical benefit.
 - Endpoint risk: multi-system disease may need a hierarchy of primary and secondary endpoints; one tissue response may not equal whole-disease benefit.
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Top 5 GT Precedent Matches

Rank	Program	Vector	Score	Confidence	Approval
1	BMN 307	AAV5	7.7/10	High	phase2
2	Hemgenix	AAV5	7.2/10	Medium	approved
3	DTX301	AAV8	7.0/10	Medium	phase2
4	Strimvelis	LV	7.0/10	Medium	approved
5	OAV101-IT	AAV9	6.9/10	Medium	approved

Match #1: BMN 307

Precedent disease: Phenylketonuria

Vector: AAV5

Tissue target: liver

Composite score: 7.7 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	1.50	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	1.00	2.0	Same or related biological pathway
Modality compatibility	1.50	2.0	Disease mechanism supports gene-addition precedent
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	0.60	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	0.20	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.80	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
Organelle targeting	1.00	1.0	Nuclear AAV delivery reaches correct subcellular compartment
TOTAL (normalised)	7.67	10.0	Raw sum / 21×10 (v2: 14 dimensions)

Rationale

- Gene CDS 1353bp / cargo 4700bp (29% utilized)
- Vector tropism plus precedent target match: liver
- Both intracellular proteins
- Inheritance match (Autosomal recessive <-> AR)
- Unknown pathway — neutral score
- Disease mechanism: loss of function — CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium

- Gene-addition modality compatibility: conditionally supports gene addition; review disease-specific constraints
- Mechanism evidence: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges
- Mechanism source: OMIM CFTR 219700 (<https://omim.org/entry/219700>)
- Approval status: phase2
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Intracellular or membrane-bound protein — no cross-correction possible; each target cell must individually receive the vector; requires high transduction efficiency and therefore a higher or more targeted dose
- Immune privilege: moderate-high privilege — tolerogenic microenvironment (Kupffer cells, IL-10, PD-L1)
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs
- ORGANELLE TARGETING: COMPATIBLE — CFTR standard subcellular localisation; nuclear AAV delivery directly produces functional protein at the correct cellular compartment with no additional organelle-import steps required.

Manual Review Flags

- Multi-system disease; define a primary therapeutic target tissue before selecting route/vector
- Vector does not naturally cover all annotated disease tissues: pancreas
- Cell-autonomous protein across multiple tissues; high transduction coverage may be required
- Narrow therapeutic window; evaluate newborn screening, presymptomatic diagnosis, or very early dosing feasibility
- Mechanism evidence is conditionally compatible with gene addition; review the listed disease-specific constraints before treating vector precedent as transferable
- Gene annotation incomplete; packaging, protein class, and localization scores need manual confirmation
- AAV tropism is species- and route-dependent; confirm human target-cell biodistribution rather than relying on animal tropism alone

Match #2: Hemgenix

Precedent disease: Hemophilia B

Vector: AAV5

Tissue target: liver

Composite score: 7.2 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue

Dimension	Score	Max	What it measures
Protein class	0.50	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	1.00	2.0	Same or related biological pathway
Modality compatibility	1.50	2.0	Disease mechanism supports gene-addition precedent
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	0.20	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.80	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
Organelle targeting	1.00	1.0	Nuclear AAV delivery reaches correct subcellular compartment
TOTAL (normalised)	7.24	10.0	Raw sum / 21×10 (v2: 14 dimensions)

Rationale

- Gene CDS 1383bp / cargo 4700bp (29% utilized)
- Vector tropism plus precedent target match: liver
- Protein class mismatch
- LOF inheritance — compatible for gene replacement
- Unknown pathway — neutral score
- Disease mechanism: loss of function — CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium
- Gene-addition modality compatibility: conditionally supports gene addition; review disease-specific constraints
- Mechanism evidence: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges
- Mechanism source: OMIM CFTR 219700 (<https://omim.org/entry/219700>)
- Approval status: approved
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Intracellular or membrane-bound protein — no cross-correction possible; each target cell must individually receive the vector; requires high transduction efficiency and therefore a higher or more targeted dose
- Immune privilege: moderate-high privilege — tolerogenic microenvironment (Kupffer cells, IL-10, PD-L1)

- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs
- ORGANELLE TARGETING: COMPATIBLE — CFTR standard subcellular localisation; nuclear AAV delivery directly produces functional protein at the correct cellular compartment with no additional organelle-import steps required.

Manual Review Flags

- Multi-system disease; define a primary therapeutic target tissue before selecting route/vector
- Vector does not naturally cover all annotated disease tissues: pancreas
- Cell-autonomous protein across multiple tissues; high transduction coverage may be required
- Narrow therapeutic window; evaluate newborn screening, presymptomatic diagnosis, or very early dosing feasibility
- Mechanism evidence is conditionally compatible with gene addition; review the listed disease-specific constraints before treating vector precedent as transferable
- Gene annotation incomplete; packaging, protein class, and localization scores need manual confirmation
- AAV tropism is species- and route-dependent; confirm human target-cell biodistribution rather than relying on animal tropism alone

Match #3: DTX301

Precedent disease: Ornithine transcarbamylase deficiency

Vector: AAV8

Tissue target: liver

Composite score: 7.0 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	1.50	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	1.00	2.0	Same or related biological pathway
Modality compatibility	1.50	2.0	Disease mechanism supports gene-addition precedent
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	0.60	1.0	Regulatory approval / trial stage
Immunogenicity	1.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	0.20	1.0	Can transduced cells rescue untransduced neighbours?

Dimension	Score	Max	What it measures
Immune privilege	0.80	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
Organelle targeting	1.00	1.0	Nuclear AAV delivery reaches correct subcellular compartment
TOTAL (normalised)	7.05	10.0	Raw sum / 21×10 (v2: 14 dimensions)

Rationale

- Gene CDS 1065bp / cargo 4700bp (23% utilized)
- Vector tropism plus precedent target match: liver
- Both intracellular proteins
- LOF inheritance — compatible for gene replacement
- Unknown pathway — neutral score
- Disease mechanism: loss of function — CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium
- Gene-addition modality compatibility: conditionally supports gene addition; review disease-specific constraints
- Mechanism evidence: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges
- Mechanism source: OMIM CFTR 219700 (<https://omim.org/entry/219700>)
- Approval status: phase2
- Vector immunogenicity (AAV8): high (~30%) — substantial patient exclusion expected; immunodepletion protocols may be needed
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Intracellular or membrane-bound protein — no cross-correction possible; each target cell must individually receive the vector; requires high transduction efficiency and therefore a higher or more targeted dose
- Immune privilege: moderate-high privilege — tolerogenic microenvironment (Kupffer cells, IL-10, PD-L1)
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs
- ORGANELLE TARGETING: COMPATIBLE — CFTR standard subcellular localisation; nuclear AAV delivery directly produces functional protein at the correct cellular compartment with no additional organelle-import steps required.

Manual Review Flags

- Multi-system disease; define a primary therapeutic target tissue before selecting route/vector
- Vector does not naturally cover all annotated disease tissues: lung, pancreas
- Cell-autonomous protein across multiple tissues; high transduction coverage may be required
- Narrow therapeutic window; evaluate newborn screening, presymptomatic diagnosis, or very early

dosing feasibility

- Mechanism evidence is conditionally compatible with gene addition; review the listed disease-specific constraints before treating vector precedent as transferable
- Gene annotation incomplete; packaging, protein class, and localization scores need manual confirmation
- AAV tropism is species- and route-dependent; confirm human target-cell biodistribution rather than relying on animal tropism alone

Match #4: Strimvelis

Precedent disease: ADA-SCID

Vector: LV

Tissue target: hematopoietic

Composite score: 7.0 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	0.30	2.0	Vector naturally reaches disease target tissue
Protein class	1.50	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	1.00	2.0	Same or related biological pathway
Modality compatibility	1.50	2.0	Disease mechanism supports gene-addition precedent
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	0.20	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.80	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
Organelle targeting	1.00	1.0	Nuclear AAV delivery reaches correct subcellular compartment
TOTAL (normalised)	7.05	10.0	Raw sum / 21×10 (v2: 14 dimensions)

Rationale

- Gene CDS 1092bp / cargo 8000bp (14% utilized)
- No tissue overlap (disease: ['lung', 'pancreas', 'liver'], vector: ['hematopoietic'])

- Both intracellular proteins
- Inheritance match (Autosomal recessive <-> AR)
- Unknown pathway — neutral score
- Disease mechanism: loss of function — CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium
- Gene-addition modality compatibility: conditionally supports gene addition; review disease-specific constraints
- Mechanism evidence: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges
- Mechanism source: OMIM CFTR 219700 (<https://omim.org/entry/219700>)
- Approval status: approved
- Vector immunogenicity (LV): low (~2%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Intracellular or membrane-bound protein — no cross-correction possible; each target cell must individually receive the vector; requires high transduction efficiency and therefore a higher or more targeted dose
- Immune privilege: moderate-high privilege — tolerogenic microenvironment (Kupffer cells, IL-10, PD-L1)
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs
- ORGANELLE TARGETING: COMPATIBLE — CFTR standard subcellular localisation; nuclear AAV delivery directly produces functional protein at the correct cellular compartment with no additional organelle-import steps required.

Manual Review Flags

- Multi-system disease; define a primary therapeutic target tissue before selecting route/vector
- Vector does not naturally cover all annotated disease tissues: liver, lung, pancreas
- No direct tissue overlap; treat this as weak precedent unless route or modality is changed
- Cell-autonomous protein across multiple tissues; high transduction coverage may be required
- Narrow therapeutic window; evaluate newborn screening, presymptomatic diagnosis, or very early dosing feasibility
- Mechanism evidence is conditionally compatible with gene addition; review the listed disease-specific constraints before treating vector precedent as transferable
- Gene annotation incomplete; packaging, protein class, and localization scores need manual confirmation

Match #5: OAV101-IT

Precedent disease: Spinal Muscular Atrophy

Vector: AAV9

Tissue target: CNS/spinal cord

Composite score: 6.9 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	1.00	2.0	Vector naturally reaches disease target tissue
Protein class	1.50	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	1.00	2.0	Same or related biological pathway
Modality compatibility	1.50	2.0	Disease mechanism supports gene-addition precedent
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	1.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	0.20	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.80	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
Organelle targeting	1.00	1.0	Nuclear AAV delivery reaches correct subcellular compartment
TOTAL (normalised)	6.90	10.0	Raw sum / 21×10 (v2: 14 dimensions)

Rationale

- Gene CDS 891bp / cargo 4700bp (19% utilized)
- Vector tropism overlaps liver, but precedent target is CNS/spinal cord
- Both intracellular proteins
- Inheritance match (Autosomal recessive <-> AR)
- Unknown pathway — neutral score
- Disease mechanism: loss of function — CFTR chloride channel dysfunction; gene addition must achieve sufficient cell-surface expression in airway epithelium
- Gene-addition modality compatibility: conditionally supports gene addition; review disease-specific constraints
- Mechanism evidence: CFTR LOF supports gene addition logic; lung delivery and transient episomal expression in cycling airway epithelium are the main challenges
- Mechanism source: OMIM CFTR 219700 (<https://omim.org/entry/219700>)
- Approval status: approved
- Vector immunogenicity (AAV9): high (~22%) — substantial patient exclusion expected; immunodepletion protocols may be needed
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Intracellular or membrane-bound protein — no cross-correction possible; each target cell must in-

dividually receive the vector; requires high transduction efficiency and therefore a higher or more targeted dose

- Immune privilege: moderate-high privilege — tolerogenic microenvironment (Kupffer cells, IL-10, PD-L1)
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs
- ORGANELLE TARGETING: COMPATIBLE — CFTR standard subcellular localisation; nuclear AAV delivery directly produces functional protein at the correct cellular compartment with no additional organelle-import steps required.

Manual Review Flags

- Multi-system disease; define a primary therapeutic target tissue before selecting route/vector
- Vector does not naturally cover all annotated disease tissues: lung, pancreas
- Only partial tissue match; verify target-cell transduction and delivery route manually
- Cell-autonomous protein across multiple tissues; high transduction coverage may be required
- Narrow therapeutic window; evaluate newborn screening, presymptomatic diagnosis, or very early dosing feasibility
- Mechanism evidence is conditionally compatible with gene addition; review the listed disease-specific constraints before treating vector precedent as transferable
- Gene annotation incomplete; packaging, protein class, and localization scores need manual confirmation
- AAV tropism is species- and route-dependent; confirm human target-cell biodistribution rather than relying on animal tropism alone